PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Is self-management a burden? What are the experiences of women self-managing chronic conditions during pregnancy? A systematic review.
AUTHORS	Jakubowski, Bethany; Hinton, Lisa; Khaira, Jaspreet; Roberts, Nia; McManus, Richard; Tucker, Katherine

VERSION 1 – REVIEW

REVIEWER	Linda Long-Bellil University of Massachusetts Medical School Department of Family Medicine and Community Health, CHPR Research & Evaluation Unit
REVIEW RETURNED	23-Aug-2021

GENERAL COMMENTS	In this article, the authors endeavored to conduct a systematic review of the qualitative literature on women's experiences of self-managing chronic conditions in pregnancy. The article was has the potential to make a meaningful contribution to the literature. My comments are below: Page 9, Line 37 – the authors refer here to patient "work." They do not explain this term until later in the article, so its meaning here is unclear. They should explain what they mean here or use a different word or phrase. Page 10, Line 28 – the wordy "synergy" does not quite work here. I understand what the authors are trying to convey, but I would suggest phrasing it differently. Page 11, Lines 7-19 – It would be helpful to explain in detail here why nearly all of the articles identified were excluded. What criteria did they not meet? Was it because they were quantitative rather than qualitative? It would also be interesting to hear what chronic conditions these other article covered. Page 12, Line 3 - supportive families, and supportive health care professionals are identified as motivators, but it seems more logical that they would serve as facilitators. Page 16, Line 26-33 – The authors state that "Diabetes was the only condition represented in the literature." However, it is unclear whether they mean all of the literature they reviewed or only the literature ultimately included in their review. Thank you for the opportunity to review this article.

REVIEWER	V Murphy University of Newcastle
REVIEW RETURNED	01-Sep-2021

GENERAL COMMENTS	This is a well written review on a subject which requires more research as the authors point out. As an asthma researcher, I was surprised there was no literature on asthma as a chronic disease in pregnancy. My only suggestions for improvement are as follows: It might be worth mentioning in the text how many of the original studies were only focused on GDM vs pre-existing diabetes. Was any difference observed in women/studies where a longer term chronic diabetes was investigated? The discussion could address other motivators and barriers found in literature from the general population (other than just "no cure").

VERSION 1 – AUTHOR RESPONSE

Reviewer: 1
Dr. Linda Long-Bellil, University of Massachusetts Medical School Department of Family Medicine and Community Health Comments to the Author:

Comment 1

Page 9, Line 37 – the authors refer here to patient “work.” They do not explain this term until later in the article, so its meaning here is unclear. They should explain what they mean here or use a different word or phrase.

Thank you, we acknowledge that those unfamiliar with this concept will need a brief definition earlier in the paper, and have given examples of patient work in this section of the paper.

Comment 2

Page 10, Line 28 – the wordy “synergy” does not quite work here. I understand what the authors are trying to convey, but I would suggest phrasing it differently.

We thank the reviewer and acknowledge this phrase can be misleading and have amended it.

Comment 3

Page 11, Lines 7-19 – It would be helpful to explain in detail here why nearly all of the articles identified were excluded. What criteria did they not meet? Was it because they were quantitative rather than qualitative? It would also be interesting to hear what chronic conditions these other article covered.

Thank you, we have added a sentence into the results to explain that the majority of articles were excluded because they were not in the correct study population, were not qualitative research, or did not include self-management.

Comment 4

Page 12, Line 3 - supportive families, and supportive health care professionals are identified as motivators, but it seems more logical that they would serve as facilitators.

We acknowledge the reviewers point that supportive families and healthcare professionals are facilitators to self-management adherence. However, the data in the included papers seemed to suggest, either implicitly or explicitly, that these networks were 'motivating' pregnant women. This language choice was slightly more reflective of the data. Respectfully, we feel 'motivators' is the appropriate choice.

Comment 5

Page 16, Line 26-33 – The authors state that "Diabetes was the only condition represented in the literature." However, it is unclear whether they mean all of the literature they reviewed or only the literature ultimately included in their review.

Thank you for pointing this out. We have amended this sentence to mean the literature included in the review.

Reviewer: 2
Dr. V Murphy, University of Newcastle

Comment 1

This is a well written review on a subject which requires more research as the authors point out. As an asthma researcher, I was surprised there was no literature on asthma as a chronic disease in pregnancy.

We thank the reviewer for their comments. There were a few articles on asthma in pregnancy, but these papers unfortunately did not include qualitative research methods.

Comment 2

My only suggestions for improvement are as follows:
It might be worth mentioning in the text how many of the original studies were only focused on GDM vs pre-existing diabetes. Was any difference observed in women/studies where a longer term chronic diabetes was investigated?

We have acknowledged in the strengths and limitations the challenges of identifying GDM vs pre-existing diabetes in the dataset, but we recognise this still remains a challenge and have included a sentence in the same section to address this comment.

Comment 3

The discussion could address other motivators and barriers found in literature from the general population (other than just "no cure").

Thank you, it is important to acknowledge the similarities between general population self-management frameworks and pregnancy self-management frameworks and we have added to the support networks section of the discussion, as we feel that the focus on social networks in the general population literature is most applicable to pregnancy.

VERSION 2 – REVIEW

REVIEWER	Linda Long-Bellil University of Massachusetts Medical School Department of Family Medicine and Community Health, CHPR Research & Evaluation Unit
REVIEW RETURNED	01-Nov-2021

GENERAL COMMENTS	This article describes the results of a systematic review of qualitative studies of women's experiences with self-management of health conditions during pregnancy. It is generally well-written and has the potential to make a meaningful contribution to the literature. My specific comments are below. Page 6, Lines 13-21 – The authors state that, “Pregnancy presents a different set of motivations for self-management, not accounted for in current self-management frameworks which often assume long-term self-management, whereas with pregnancy the period of management is temporarily bounded even if it continues post-partum and considerations are for the mother and baby dyad.” Assuming that the authors intend to include women with chronic conditions (diabetes Type I or Type II), rather than limit the focus to women with conditions specific to pregnancy (gestational diabetes), I would first suggest that it would be more accurate to state that pregnancy presents an additional set of motivations, rather than a different set of motivations. I think it is unlikely that women’s pre-pregnancy motivations would completely disappear, but rather these concerns would recede into the background as the health of the baby takes precedence. In this same vein, I think that the authors should specify that the themes they explore later in this paper are only those related to pregnancy. Second, I would add the words “exists before pregnancy and” after the words “even if it” I think this would clarify the authors’ intended meaning. Page 7, Lines 36-39 – The authors state that, “It was not appropriate or possible to involve patients or the public in the design, or conduct, or reporting, or dissemination plans of our research.” I realize that including "patients" is not typically done in systematic reviews, but I think that saying that is not "appropriate or possible" is going a bit too far. Page 8, Line 22 – I would suggest putting the term “sick people” in quotation marks as this term may be offensive to some readers and it would behoove the authors to make clear that this term comes from the author who originally devised the Burden of Treatment theory. Page 8, Lines 18-36 – If there is a diagram or other type of figure that could be used to illustrate the Burden of Treatment theory, that would be helpful here. Page 14, Lines 20-35 – Clarify this language to make clear whether the authors are including women with chronic diabetes (Type I or Type II) along with those with gestational diabetes. The language here is not entirely clear.

VERSION 2 – AUTHOR RESPONSE

Reviewer Comment 1

Page 6, Lines 13-21 – The authors state that, “Pregnancy presents a different set of motivations for self-management, not accounted for in current self-management frameworks which often assume long-term self-management, whereas with pregnancy the period of management is temporarily bounded even if it continues post-partum and considerations are for the mother and baby dyad.”

Assuming that the authors intend to include women with chronic conditions (diabetes Type I or Type II), rather than limit the focus to women with conditions specific to pregnancy (gestational diabetes), I would first suggest that it would be more accurate to state that pregnancy presents an additional set of motivations, rather than a different set of motivations. I think it is unlikely that women’s pre-pregnancy motivations would completely disappear, but rather these concerns would recede into the background as the health of the baby takes precedence. In this same vein, I think that the authors should specify that the themes they explore later in this paper are only those related to pregnancy. Second, I would add the words “exists before pregnancy and” after the words “even if it” I think this would clarify the authors’ intended meaning.

Author response

We thank the reviewer for this comment and have amended the wording, changing ‘different’ to ‘additional’ as suggested. While we have not used the specific wording suggested here, we have added an additional sentence in the discussion to clarify that some of the barriers and motivations identified in the review exist in the management of chronic conditions outside of pregnancy.

Reviewer Comment 2

Page 7, Lines 36-39 – The authors state that, “It was not appropriate or possible to involve patients or the public in the design, or conduct, or reporting, or dissemination plans of our research.” I realize that including "patients" is not typically done in systematic reviews, but I think that saying that is not "appropriate or possible" is going a bit too far.

Author response

The authors acknowledge that this comment was perhaps not appropriate and in response to this comment, and the editor’s note, we have removed this from the paper and updated our PPI statement.

Reviewer Comment 3

Page 8, Line 22 – I would suggest putting the term “sick people” in quotation marks as this term may be offensive to some readers and it would behoove the authors to make clear that this term comes from the author who originally devised the Burden of Treatment theory.

Author response

The authors agree that this would be beneficial for an audience unfamiliar with the theory and thank the reviewer for this suggestion.

Reviewer Comment 4

Page 8, Lines 18-36 – If there is a diagram or other type of figure that could be used to illustrate the Burden of Treatment theory, that would be helpful here.

Author response

While we understand and appreciate why the reviewer has requested a figure to illustrate the burden of treatment theory here, we feel it would not be appropriate to include one in this instance as it would not be our own work. May et al’s paper on the burden of treatment theory has several useful illustrations of the theory (reference 14 in the paper), but as we have only used this theory as a theoretical lens to understand the data and not developed the theory itself, we did not feel it was appropriate to create our own illustrative figure and would, instead, guide people towards the figures in May et al’s paper.

Reviewer Comment 5

Page 14, Lines 20-35 – Clarify this language to make clear whether the authors are including women with chronic diabetes (Type I or Type II) along with those with gestational diabetes. The language here is not entirely clear.

Author response

We thank the reviewer for this comment and we have added in that we are referring to all types of diabetes in this instance for clarification.